



February 07, 2024

Assessment of Pharyngitis

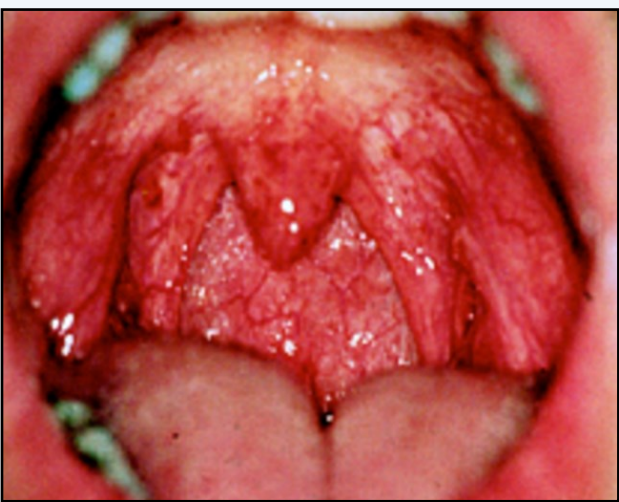
Clinical Case Quiz

Procedures for Suspected Strep Throat

A 26-year-old male calls with a two-day history of sore throat, fever, and body aches. He has no cough, can swallow his own secretions, and reports getting strep throat every year. He says he just needs some antibiotics. You obtain a photograph of his throat over patient picture request and receive the picture shown on the right.

He hasn't had any known exposure to strep. You recommend he go to his local medical office building to get tested for strep. If the rapid strep is negative, you order a throat culture to be done. You should also consider ordering the following:

- a. Steroids
- b. Antibiotics, but instruct the patient to pick them up only if he tests positive for strep
- c. Rapid COVID and flu testing
- d. PCR COVID and flu testing
- e. 3 of the above



Note. From Pharyngitis [Photograph], by UpToDate, n.d., UpToDate (<https://www.uptodate.com/contents/symptomatic-treatment-of-acute-pharyngitis-in-adults>)

The answer is d.

The Current Strep Workflow

Best Practices for Diagnosing and Treating Pharyngitis

COVID and influenza can also present with pharyngitis, especially in the presence of fever and myalgia. Please **do not** order rapid COVID and flu testing over a telehealth encounter.

You no longer need to order antibiotics if you send a member for strep testing to nurse clinic. If the test is positive, then the local Doctor of the Day will provide orders for treatment. If the rapid test is negative and subsequent throat culture is positive, then treatment will need to be called in either by you or a provider covering your InBasket.

If a strep test is pending, avoid providing treatment with antibiotics if you've used the following diagnostic codes:

- Acute Pharyngitis J02.9
- Acute Tonsillitis J03.90
- Pharyngitis J02.9
- Throat Pain R07.0
- Throat Irritation J39.2
- Tonsillitis J03.90

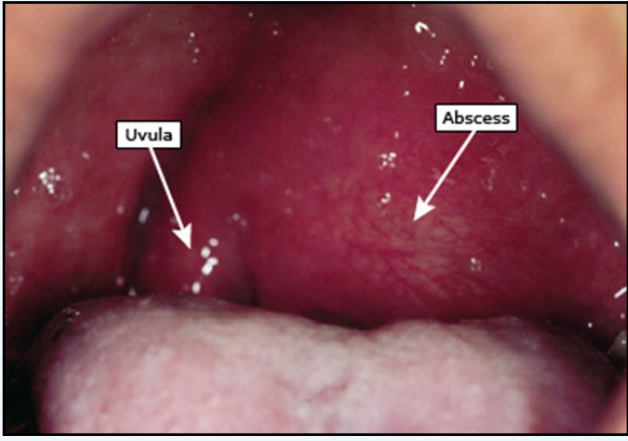
In general, it is best to avoid ordering steroids over telehealth or in the outpatient setting for pharyngitis that is mildly to moderately painful.

Supportive care measures for sore throat include saltwater gargles; over-the-counter medications, such as Tylenol and NSAIDs; and hydration.

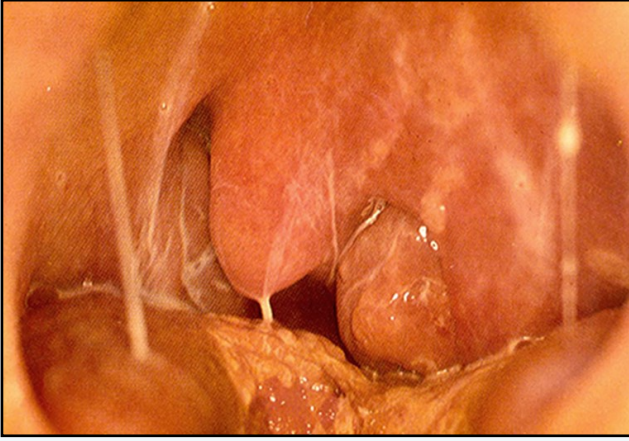
The rapid strep protocol is appropriate for members who do not have symptoms of a peritonsillar abscess. Peritonsillar abscess presents with a severe sore throat, a muffled "hot potato" voice, and an inability to swallow secretions. Trismus or difficulty opening the mouth can also be present.

It is of paramount importance to assess a patient's airway if there is concern for peritonsillar abscess. They should be seen promptly either in the urgent care or in the ER. Additionally, patient picture request can be used to assess a patient's throat. Uvular deviation to the opposite side and a bulging of the posterior soft palate of tonsil are hallmarks of peritonsillar abscess.

Below are examples of presentations for peritonsillar abscess.



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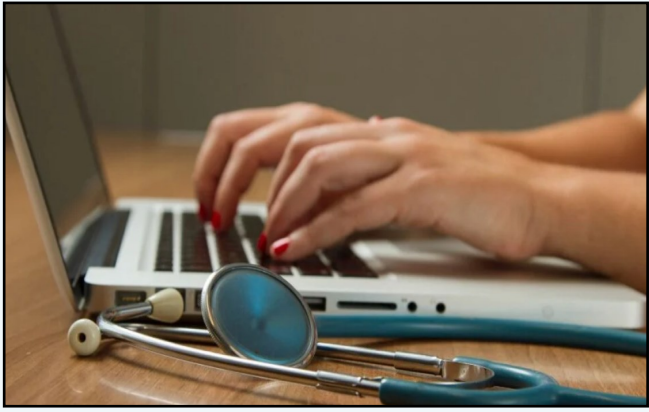


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Completing a Patient Picture Request

For more information on patient picture request, [please view the job aid](#).

Introducing the GPC Mailbag!



At GPC Clinical Corner, we strive to provide tips on the best clinical practices, navigating patient encounters, and updated information on treatment options.

This year, we'd like to expand our scope to include your suggestions. We're proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you'd like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

UpToDate. (n.d.). Pharyngitis [Photograph]. UpToDate. <https://www.uptodate.com/contents/symptomatic-treatment-of-acute-pharyngitis-in-adults>